PUBLIC TRANSLATION ARTIFACT · 01 IN READING ORDER

NHS

/ PRIMARY CARE

PLAIN-LANGUAGE COMPANION

For leadership, clinical teams, policymakers, patients, carers, and builders.

> *"NHS primary care is record-rich and continuity-poor.*
> *Too often, the patient becomes the record."*

ARTIFACT FAMILY

#	ARTIFACT	ACCESS
01	**Translation Artifact**	← *This document*
02	Diagnostic Artifact	Public
03	Redesign Executive Summary	Public
04	Full Redesign Artifact	Restricted

PREPARED BY Jamie Forrester · Independent Systems Architect · Edinburgh, UK
CONTACT hello@jamieforrester.com · jamieforrester.com
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CLASSIFICATION Public · 01 in reading order · start here

START HERE — READING ORDER

Translation Artifact (start here) → Diagnostic Artifact (full evidence) → Redesign Executive Summary (replacement logic) → Full Redesign Artifact (restricted builder specification). Use this document to create shared understanding. Use the Diagnostic for evidence. Use the Full Redesign for implementation.

Quick reference

ITEM	SUMMARY
The problem	NHS primary care has no reliable structural mechanism to ensure operational memory is present before clinical decisions are made. Too often, the patient becomes the record.
The failure	The GP consultation collapses four system functions into one: memory reconstruction, sensing, interpretation and clinical authority. This is function fusion, not just heavy workload.
What must become true	Five conditions: continuity named as a requirement; an operational record separate from the audit record; usable history in front of any clinician before the decision; outcomes and handovers carried forward; and continuity that holds when the patient cannot carry it.
The proposed route	A patient-held operational continuity record. History, symptom events and a structured consultation packet exist before the consultation. The clinician receives the case; they do not have to reconstruct it.
The 7 moves	1. Truth kernel → 2. Symptom event layer → 3. Consultation packet → 4. Outcome append → 5. Emergency access → 6. Carer access → 7. NHS recognition pathway
Proof	Same pressure → different response. Patients no longer repeat their history as the default. The GP begins at clinical judgment. Continuity holds even when the patient cannot tell the story.
Biggest risk	Recapture: custody pulled back into institutions, or the app turning into an audit tool or health journal. Patient custody and decision-ready retrieval must be preserved.

Plain-language terms used in this document

TERM	PLAIN MEANING
Operational memory	The usable clinical picture that must be present before the next decision is made. Not a record of the past — the context needed right now.
Hidden burden (ghost structure)	The hidden human work — patients repeating history, carers keeping diaries, GPs reconstructing cases — that the architecture should be doing but is not.
Collapse node	The point where too many system failures converge in one place at one moment. Here: the GP consultation.
Function fusion	Several structurally different jobs compressed into one role at the same time. The GP sensing, interpreting, deciding and remembering at once is function fusion.
Continuity record	The patient's portable operational memory: history, medications, symptom events, clinical outcomes and pending follow-ups — in one place, held by the patient.
Consultation packet	The one-page structured summary prepared before a GP encounter, readable in under 60 seconds. This is what the redesign delivers to the clinician instead of a cold start.

TERM	PLAIN MEANING
Recapture	The old failure returning inside the new system. When the app becomes a health journal, or an institution takes custody of the record, recapture has occurred.
Append-only	You can only add to the record. Nothing is deleted or overwritten. Corrections are added alongside the original so the history stays visible.
Vanish List	The refusal conditions that reproduce the failure. Behaviours stop immediately; structures stop before their replacement becomes the official path; nothing scales while one is still active.

SECTION 0

WHAT THIS DOCUMENT IS

This Translation Artifact explains the structural diagnosis and the replacement path in plain language. It is not a policy document, a reform proposal or a product specification. It is a companion to the Diagnostic and Full Redesign Artifacts: it introduces no new claims and makes no independent evidence assertions. Its job is translation — to make the diagnosis and the replacement understandable without requiring the reader to enter the full evidence base or the restricted build specification.

It preserves the judgment boundary of the Diagnostic Artifact: this is a structural reading from public evidence, not a claim about undisclosed internal systems, intent, or individual failure.

What this document does not say

- → It does not say GPs are failing. It says the architecture places structural demands on the GP consultation that it was not built to carry.
- → It does not say the 10-minute appointment must be abolished. It says the consultation should begin at clinical judgment, not reconstruction.
- → It does not say all patients must see the same GP. It says operational memory must be present whichever clinician they see.
- → It does not say the app is a diagnostic tool, a medical device or a clinical decision system.
- → It does not say the app replaces the official NHS GP record.
- → It does not say NHS IT cooperation is required before the redesign delivers value.
- → It does not claim patient-facing records are new — platforms such as Patients Know Best and the NHS App already exist. What the redesign specifies is how continuity must arrive at the decision.
- → It does not claim access to unpublished NHS internal data.

SECTION 1

THE SYSTEM IN ONE SENTENCE

GOVERNING DIAGNOSIS

NHS primary care operates without reliable structural memory at the point of care, forcing the GP consultation to collapse sensing, interpretation, history reconstruction and clinical authority into a single time-constrained event.

SECTION 2

WHAT THE SYSTEM SAYS VS WHAT IT DOES

The NHS promises continuity of care: joined-up records, informed decisions, and the right information at the right time. In practice, continuity is often missing before the consultation begins.

> ***The patient supplies history. The carer supplies context. The GP reconstructs the case. The system supplies fragments.***

A patient arriving after a hospital stay may find the discharge summary has not arrived. The GP has no complete picture. The patient becomes the record.

The system promises memory. The consultation performs reconstruction.

The NHS also records the patient as a series of events — an appointment, a referral, a discharge. But the patient is not an event. Take a patient with diabetes who has three GP appointments, one hospital visit and two prescription changes over six months. Each event is recorded separately. The patient's condition exists continuously between them. The current system has no reliable structural way to connect those dots across care settings. That is why the patient keeps repeating their history: they are often the only thing linking the events together. The system does not reliably remember across encounters unless a human carries the memory.

This is not a clinician failure. It is a structural mismatch between promise and architecture.

SECTION 3

WHERE THE BURDEN LANDS

The system appears to function because people carry work the architecture does not. In systems terms, this hidden layer of unrecognised, uncompensated labour is the ghost structure.

- → **Patients carry memory:** history, symptoms, medication context, prior events, and the clinical picture the system does not reliably retrieve.
- → **Carers carry continuity:** logs, timelines, medication diaries and transition details across care settings.
- → **GPs carry fused clinical burden:** sensing, interpretation, memory reconstruction and authority inside the consultation — at the same time, under time pressure.
- → **Receptionists and care navigators carry informal routing:** complexity flagging and continuity brokering that the system does not formally assign to anyone.

The carer's health notebook is the ghost structure made physical. The notebook that travels between GPs, hospitals and specialists — holding the medication changes, the discharge dates, the symptom timeline — is not a lifestyle preference. It is operational memory the architecture has not held. When we say the redesign succeeds when the carer's diary is no longer needed, we mean it: the notebook is visible proof of what the system is not doing.

> STRUCTURAL READING
>
> ***These are not support roles. They are the missing system. Remove them, and the continuity gap becomes visible. The redesign succeeds when these carriers are no longer structurally necessary.***

SECTION 4

THE COLLAPSE NODE

The GP consultation has become the collapse node: the point where the system's structural failures converge in one place, at one moment, under full clinical consequence.

In a short appointment, the clinician must at once: reconstruct history from whatever the patient can remember; sense presenting symptoms against an incomplete record; interpret those signals into a clinical picture; make a consequential clinical decision; and document the encounter.

> ***That is not simply heavy workload. It is function fusion: four distinct system functions compressed into one person at the exact point of clinical consequence. The system's structural gap is routed directly into the consultation.***

FUNCTION	WHERE IT SHOULD BE	WHERE IT ACTUALLY IS
Sensing	Upstream — before the patient arrives	Inside the consultation — the GP absorbs symptoms as the patient speaks
Interpretation	Bounded by prior context — available before judgment	Fused with authority — the GP interprets and decides at the same moment
Authority	The GP consultation, acting on structured prior state	The GP consultation, acting on whatever was just reconstructed
Memory	The system — present before the encounter	The patient — reconstructed verbally at the start of the appointment

SECTION 5

COST OF CONTINUATION

The current structure compounds cost and risk as demand rises. Clinical time is spent reconstructing what should already exist. Diagnostic delay becomes normal. Repeat appointments increase. Clinical risk rises at each handover. Workforce strain adds to burnout and exit.

> ***Scaling access without fixing continuity does not solve the problem. It sends more patients through the same collapse node.***

Public evidence shows the scale of the environment in which this pattern appears:

- → NHS Resolution paid about £2.8 billion in clinical negligence claims across all its schemes in 2023/24 — an NHS-wide figure, mostly hospital claims.
- → The Guardian reported 36 coroners' warnings in 2024 concerning the sharing of NHS patient information.
- → 42% of GPs said they were unlikely to be working in general practice in five years' time (RCGP, 2024).

These figures are drawn from the Diagnostic Artifact and its cited public sources (NHS Resolution Annual Report 2023/24; The Guardian, November 2024; RCGP, September 2024). This Translation Artifact introduces no independent quantitative

claims. The figures do not show that these costs are caused by the continuity failure. They show the scale of the consequence environment in which the same information-sharing and continuity pattern repeatedly appears.

SECTION 6

WHY PREVIOUS FIXES HAVE NOT HELD

The NHS has added more records, more portals, more patient access features and more digital programmes. Yet one question remains unanswered at the consultation:

> ***Is usable clinical continuity present before the decision is made?***

A record is not memory. Access is not continuity. Documentation is not retrieval.

The NHS has built audit trace — a record of what happened, produced for compliance and defensibility. What it has not reliably built is operational memory — the clinical picture that must be present before the next decision. The programmes reviewed in the Diagnostic improved the audit and access layers; none was measured by whether usable history reached the decision.

The proposed Single Patient Record is the next programme in this line. The test for it is the same: is it measured by data assembled, or by usable continuity at the point of decision? As long as memory is not structurally present before the consultation, new digital layers remain liable to relocate the same failure under a new name.

SECTION 6.1

WHAT MUST BECOME TRUE

The Diagnostic Artifact sets out five conditions, in dependency order. They do not depend on any particular technology.

1. **Continuity named as a requirement** — who holds it, when it must be present, and the minimum standard.
2. **An operational record separate from the audit record** — built for retrieval at the next decision, not for compliance.
3. **Usable history in front of any clinician before the decision** — including repeat unresolved symptoms.
4. **Outcomes and handovers carried forward** — consultation outcomes, discharge and referral information reach the next decision.
5. **Continuity that holds when the patient cannot carry it** — emergencies, carers, and digitally excluded patients.

The rest of this document describes the route proposed in the Full Redesign Artifact for meeting these conditions: a patient-held operational continuity record that can start working without waiting for institutional change.

SECTION 7

THE REDESIGN IN PLAIN LANGUAGE

The redesign separates the four functions and gives each one a proper structural home. It does not ask the GP to do less. It asks the architecture to do more.

Before and after

FUNCTION	BEFORE — CURRENT STATE	AFTER — TARGET STATE
Memory	Patient repeats history at appointments; GP personal recall for known patients only	Patient-held continuity record, present before the encounter and readable by any clinician
Sensing	GP absorbs presenting symptoms while reconstructing prior state	App captures symptom events upstream; structured prior context is available before the encounter
Structured preparation	None — GP opens the encounter cold	App prepares the consultation packet: symptom event, relevant history, medications, red flags, pending follow-ups
Authority	GP senses, interprets, reconstructs and decides at once, under time pressure	GP receives structured prior state; exercises clinical judgment; the outcome is appended

CORRECTED PRIMITIVE

Operational continuity must be structurally present before the clinical decision, in patient-held form. Clinical authority remains with the clinician.

Before the consultation, the system must carry: **memory** (history, medication context, prior events, unresolved patterns); **structured sensing** (symptom events captured upstream); and **structured preparation** (the consultation packet).

Inside the consultation, the clinician keeps full authority: clinical judgment, diagnosis, prescribing, referral and treatment decisions. The GP should not have to begin by reconstructing the case. The GP should receive the case.

This is not a better notes system. It is a reallocation of system functions.

Why patient-held? The patient is the only thing present at every encounter, in every setting. A patient-held record can start working now, without waiting for systems to connect, and it crosses every organisational boundary with the person. An institutional route — such as the proposed Single Patient Record — could also meet the five conditions, but only if it is measured by usable continuity at the decision and does not push the work back onto the patient. The Full Redesign Artifact sets out the full case (Section 5.1).

SECTION 7.1

WHAT THE MINIMUM REDESIGN REQUIRES

The redesign is only real if five components exist together. A deployment missing any one of them is not a replacement. It is a partial correction inside the old frame.

1. **A patient-held operational continuity record** — portable, persistent, and retrievable at any point of care by any clinician. If the record is held by an institution rather than the patient, the redesign has not begun.
2. **A symptom event layer** — structured symptom capture before the encounter, creating the information the consultation packet draws on. Routing prompts are an optional add-on, switched on only once their regulatory classification is settled.
3. **A consultation packet readable in under 60 seconds** — prepared before the encounter from the continuity record. If the GP still begins by reconstructing history verbally, the packet is not doing its job.

4. **Outcome append after every consequential encounter** — the clinical outcome, prescription, referral and follow-up are recorded and travel to the next consultation. If outcomes disappear after encounters, continuity is not building up.
5. **Emergency access** — essential fields available without login to first responders, including when the phone is unavailable. If the system fails in the hardest case, it has not been fully built.

Two inclusion requirements apply across all five. **Carer access** — delegated access with explicit role tagging — and **non-digital fallback** — PDF export, a printed packet and a physical Continuity Card — so that digitally excluded patients receive the same benefit. The **NHS recognition pathway** is Move 7: a later migration step, not a component the redesign needs in order to be useful.

> **MINIMUM REPLACEMENT RULE**
>
> ***If any one of the five components is missing, the change is not a redesign. It is a partial correction inside the old frame — and the old failure remains liable to return in a new interface.***

SECTION 8

THE REDESIGNED OBJECT

The redesign centres on a patient-held operational continuity record. In the proposed redesign, its first carrier is a continuity app, supported by PDF export, emergency access and carer access. It is not another static record. It is a preparation and memory system designed to make the patient's prior clinical state available before the consultation begins.

At setup

The patient records core continuity information: identity, GP and hospital contacts, confirmed diagnoses, current medications, known allergies, prior events, emergency contacts and carer details. Minimum setup must take under 7 minutes. The record becomes more complete as later encounters are appended. The patient should spend less time on the app than they currently spend repeating their history.

When symptoms occur

The patient records symptom events in structured form. The app organises them into a timeline and surfaces recent changes, unresolved episodes, pending follow-ups and known risks already recorded in the patient's own record.

The app does not diagnose, triage, prescribe, refer or hold clinical authority. It organises information so the clinician can judge. Routing prompts, where used, are an optional feature released only after their regulatory classification is settled. They are bounded navigational aids. They do not detect conditions, rank risks, reassure the patient or make clinical assessments. They only help the patient choose a route for seeking care, based on information the patient entered.

> **SAFETY INFORMATION**
>
> For patients and carers: routing prompts in the app are not medical advice. They are based on information you enter. They do not replace a clinician's judgment. If you are worried about your symptoms, seek medical attention — do not wait for the app to tell you that you are safe. The app is a continuity tool, not a diagnostic service.

Before the consultation

The app prepares a consultation packet: the current symptom event, relevant history, recent clinical outcomes, active medications, pending follow-ups, known risks and unresolved patterns. It is decision-ready by design — the GP is not looking at a list of symptoms but at a symptom-to-history bridge that has already organised the prior state.

> **THE 60-SECOND RULE**
>
> The packet must be readable in under 60 seconds. If a clinician who has never seen the patient cannot begin clinical judgment within 60 seconds of opening the packet, the packet has failed. Sixty seconds is not a speed target — it is a structure test. It marks the boundary between a data dump and a minimum viable clinical picture: the prior state a GP needs to act, not everything that has ever happened.

After the consultation

The outcome is appended to the continuity record: clinical decision, prescription, referral, advice, follow-up instruction, or no action. This closes the loop. Each appended outcome makes the next consultation better prepared.

Who wrote what is always visible. Outcomes may be entered by the patient, a carer or the clinician, and every entry is tagged with who entered it and when. A GP reading the packet can see what is clinician-entered and what is patient-reported. The official NHS GP record remains the medico-legal record. The clinician still judges. The packet prepares the judgment.

Offline access. The continuity record and consultation packet must be available on the device without an internet connection — in a hospital basement, or a rural area without signal. The physical Continuity Card — credit-card sized, printable, with a QR code to the emergency view — gives access even if the phone is unavailable, uncharged or damaged.

The app does not replace the clinician. It is designed to take the reconstruction work out of the consultation before the clinician enters the decision.

SECTION 8.1

PATIENT-HELD ≠ PATIENT-BURDENED

> **PATIENT-HELD ≠ PATIENT-BURDENED**
>
> ***Patient-held does not mean patient-burdened. The patient holds custody of the continuity record. The architecture must reduce the labour required to maintain, retrieve and present it. If the app increases patient administrative burden while calling it empowerment, the redesign has failed.***

Today, the patient carries continuity as hidden labour: repeating history, keeping medication lists, bringing handwritten notes. The redesign replaces that labour with custody. The patient holds the record; the system does the work of organising, surfacing and presenting it. Whether the burden actually falls is measured, not assumed.

The three-part allocation: the patient holds custody. The system carries memory and preparation. The clinician carries authority.

SECTION 9

THE MOVE ORDER

The redesign must happen in sequence. Right fixes in the wrong order recreate the failure. Each move depends on the ones before it; Moves 5 and 6 both build on Moves 1–4 and can proceed side by side. Preparatory work may begin early; nothing becomes the official path or scales before its dependency is complete.

MOVE	WHAT CHANGES	WHY IT COMES HERE
1 — Truth kernel	Build the patient-held continuity record and the onboarding flow. Minimum setup under 7 minutes; extended fields added over time. Privacy and security foundations (Move 1b) follow directly.	The foundation. Nothing else can be built on an undefined continuity object.
2 — Symptom event layer	Build structured symptom capture before the encounter	Moves sensing upstream; creates the information the packet draws on
3 — Consultation packet	Build the one-page packet for the consultation, readable in under 60 seconds	The core mechanism. Addresses the first structural break directly.
4 — Outcome append	Build post-consultation outcome recording into the continuity record	Closes the loop; lets continuity build up across encounters
5 — Emergency access	Build lock-screen access to essential fields, plus the physical Continuity Card	Handles the hardest case: when the patient cannot tell the story
6 — Carer access	Build delegated access with explicit role tagging	Supports digitally excluded and low-literacy patients; names the carer role formally
7 — NHS recognition	Open a formal pathway for NHS ingestion without custody transfer (the One-Way Valve)	Deferred until the app has shown value in real use; NHS integration is Move 7, not Move 1

The full specification for each move — completion criteria, rollback conditions and validation tests — is in the Full Redesign Artifact, Section 10.

SECTION 10

WHAT MUST STOP

Six things must stop. They are refusal conditions, not optimisation targets. Behaviours that reproduce the failure stop immediately. Structures that reproduce it stop before their replacement becomes the official path. Nothing scales while a refusal condition is still active.

STOP	WHY	WHEN
Treating the patient as the default memory carrier without structural support	The patient performing the memory function is the first structural break. Building on that assumption replicates the break in a new interface.	Immediately

STOP	WHY	WHEN
Measuring success by engagement or adoption rather than continuity retrieval at the point of decision	Downloads, sessions and daily use reward documentation volume. The test is whether usable continuity is present when a clinical decision is made. An app designed for daily engagement becomes a health journal.	Immediately — before any metric framework is set
Presenting routing output as diagnosis or condition detection	The app must never become a diagnostic authority. Detecting conditions, ranking risks or reassuring patients that care is unnecessary moves it into a different, regulated product class.	Immediately — in any interface, marketing or communication
Treating NHS integration as a prerequisite for usefulness	If the app only works with NHS integration, adoption needs institutional permission. The route depends on being useful first.	Immediately — before the roadmap is set
Adding features that improve the audit record without improving retrieval	Features that add documentation without changing whether continuity arrives at the decision repeat the old correction pattern. PDF export is a fallback, not the product.	Immediately — an ongoing design gate
Treating the app as the official NHS record or a replacement for it	Before formal recognition (Move 7), the app is a patient-held continuity layer, not the medico-legal record. Confusing the two creates clinical and regulatory risk.	Immediately — before any public claim of clinical equivalence

These six are the implementation Vanish List in the Full Redesign Artifact, Section 8. The Diagnostic Artifact also names four system-level refusal conditions — for example, treating discharge-summary failure as a communication problem.

SECTION 11

WHAT THE SYSTEM MUST NOW CARRY ITSELF

The redesign moves load-bearing continuity work out of people and into explicit system functions. It succeeds only when that burden actually falls — which is measured, not assumed.

CURRENT BURDEN	MUST BECOME
Patient repeats their history at appointments	Structured patient-held continuity record, present before the encounter
Carer keeps separate medication logs and appointment diaries	Delegated continuity access; outcome append captures the post-encounter record
GP reconstructs the case from fragments and patient recall	Decision-ready consultation packet prepared before the encounter
Receptionist routes without structured context	Structured symptom context from the symptom event layer
Repeat unresolved symptoms visible to no one	Repeat presentations surfaced in the continuity record and packet
Clinical outcome disappears after the encounter	Outcome appended to the continuity record; appears in later packets

If the GP is still reconstructing the case, the patient is still the memory system, or the app has become another static record, the redesign has not succeeded.

SECTION 12

WHAT SUCCESS LOOKS LIKE

The redesign should change lived experience before it changes institutional metrics. These are target states, to be tested against baseline measures.

AUDIENCE	BEFORE	TARGET STATE
Patient	Repeats history at appointments; brings handwritten notes; cannot assert history the system does not have	Arrives with the continuity record; no longer reconstructs history verbally; leaves with a record of what was decided
Carer	Keeps a separate medication diary; brings appointment logs; acts as the main memory bridge across transitions	App holds the outcome from each encounter; no parallel diary needed; continuity is no longer the carer's burden to organise
GP	Opens encounters cold; reconstructs from fragments and recall; clinical judgment squeezed into whatever time is left	Reads the consultation packet in under 60 seconds; begins at clinical judgment; the outcome is appended afterwards
NHS leadership	Diagnostic delay, negligence exposure, GP exit pressure, and repeated coroner concern about information sharing	Measured reduction in GP reconstruction time and history repetition; hidden carriers no longer structurally necessary

SECTION 13

WHAT PROVES THE REDESIGN IS REAL

> PROOF CONDITION
>
> **Same pressure → different response.** The redesign is real when the same clinical workload and patient complexity produce a different encounter: the patient does not repeat their history, the GP begins with the case already structured, and continuity holds even when the patient cannot tell the story.

Before a pilot begins, baselines are captured: history repetition, GP time on reconstruction, discharge information present before follow-up, and carer diary maintenance. Without a baseline, same pressure → different response cannot be shown.

The redesign is confirmed when all of the following hold:

- → Patients no longer routinely repeat their full history at each encounter
- → For patients using the redesigned route, consultations start with structured context rather than reconstruction
- → Repeat unresolved presentations are surfaced in the packet before the encounter begins
- → Outcomes persist and build up across encounters
- → Continuity holds when the patient cannot tell the story — unconscious, acutely distressed or cognitively impaired
- → A clinician who has never seen the patient can understand the relevant prior state in under 60 seconds

- → Carers no longer keep separate health diaries as the main continuity mechanism
- → High-burden patients — repeat presenters, post-discharge patients, people with several long-term conditions — benefit, not only digitally confident early adopters

When these conditions hold against the baseline, memory has moved into structure.

SECTION 13.1

WHAT IT LOOKS LIKE IN PRACTICE

Before the redesign

A patient is discharged from hospital after a short stay. A week later they attend a GP appointment. The discharge summary has not arrived. The GP asks the patient to explain what happened, what medication changed, what the hospital decided and what follow-up was planned. The patient remembers some of it. They cannot recall the exact medication change. They did not bring the letter. The GP pieces together a clinical picture from fragments and makes a decision on partial information.

After the redesign

The same patient attends the same appointment. Before the consultation begins, the GP opens the consultation packet. It shows the discharge event, the medication change, the pending follow-up, the symptom the patient logged yesterday, and the last recorded outcome — each tagged with who entered it. The GP reads it in under 60 seconds. The appointment begins with the case already assembled. The GP still judges. The GP still examines. The GP still decides. But they start from an organised picture rather than building one from scratch.

> ***The GP's job does not change. The quality of the input to that job does.***

SECTION 14

FAKE PROGRESS — HOW TO KNOW IF IT ISN'T WORKING

The redesign can be implemented in appearance while the old failure continues in structure. These are signs that the primitive has returned, not that the redesign has succeeded:

SIGN	WHAT IT MEANS
The app exists but patients still repeat their full history	The app is a health journal, not a continuity system. The packet is not changing the encounter.
Clinicians do not read the packet; appointments still begin with reconstruction	The 60-second rule has failed. The packet is a document, not a mechanism.
Success is measured by downloads and engagement rather than continuity retrieval	Growth metrics reward the audit layer. The test is retrieval before the clinical decision.
The record becomes institution-owned under NHS integration	The custody invariant is broken. The broken architecture has been rebuilt in a new wrapper.
The app presents symptom patterns as clinical diagnoses	The app has entered clinical authority it cannot hold, with medical-device classification risk.
Carers are still keeping separate health diaries	The hidden burden has not dissolved. It has not moved from people into structure.

SIGN	WHAT IT MEANS
The redesign is declared complete before baselines were taken	Without a baseline, same pressure → different response cannot be shown.

SECTION 15

RECAPTURE WARNING

The greatest risk after the redesign is built is recapture: the old failure quietly reinstalling itself under pressure. Under pressure, the system can drift toward:

- → **Pulling custody back into institutions** — requiring the continuity record to be held by the NHS rather than the patient
- → **Converting continuity into audit documentation** — optimising the record for compliance and defensibility rather than clinical retrieval
- → **Reintroducing reconstruction** — letting the packet become optional, ignored, or replaced by verbal history
- → **Measuring the app by engagement** rather than continuity retrieval — turning it into a health journal

RECAPTURE PRINCIPLE

The patient-held continuity record is not a preference. It is the corrected primitive. The redesign survives only if patient custody survives, retrieval stays decision-ready, and the four functions stay separated.

Coercive access is also a recapture risk. Patient-held health records create risks that institution-held records do not: a family member, partner, carer, employer or institution may pressure a patient to share sensitive health information. Patient-held custody only works if access can be revoked at any time, sensitive information can be hidden from the emergency view, and the patient can see a full history of who has accessed the record.

SECTION 16

HOW TO USE THIS DOCUMENT

Use this Translation Artifact before the full artifacts, not instead of them.

AUDIENCE	USE THIS DOCUMENT TO…	THEN GO TO…
Leadership and policymakers	Understand the structural failure, the cost of continuation, and the direction of the replacement	Redesign Executive Summary (Artifact 03) · public board summary
Clinical teams	See what burden must leave the consultation, and what the consultation packet changes for the encounter	Diagnostic Artifact (Artifact 02) · full evidence base
Patients and carers	Understand why history repetition is structural, not personal, and what the redesigned route changes. For direct patient-facing use, produce a one-page summary drawn from Sections 3, 8, 12 and 13.	This document is the starting point; share Sections 3 and 8 directly with patients

AUDIENCE	USE THIS DOCUMENT TO…	THEN GO TO…
Builders and product teams	Use the move order as orientation, and the Full Redesign Artifact for build detail	Full Redesign Artifact (Artifact 04) · full technical specification, schemas, invariants and testability

SECTION 17

ONE-PAGE LEADERSHIP SUMMARY

Five questions a board member needs answered

LEADERSHIP QUESTION	ANSWER
What is failing?	Operational memory is not reliably present at the point of care. Too often, the patient has become the record. The GP consultation collapses four system functions into one person under time pressure.
Why have previous fixes not held?	They improved records and access, not decision-ready retrieval. The NHS has built extensive audit trace; it has not reliably built operational memory. New digital layers remain liable to relocate the failure rather than correct it.
What must replace it?	Five conditions must become true. The proposed route: a patient-held operational continuity record, with a consultation packet readable in under 60 seconds. The patient holds custody. The system carries memory and preparation. The clinician carries authority.
What must not happen?	The patient left as the memory carrier; engagement metrics in place of retrieval; diagnosis by app; NHS integration as a prerequisite; audit features with no retrieval gain; the app treated as the official record.
What would prove it worked?	Same pressure → different response, against baseline: no routine history repetition; GP reads the packet in under 60 seconds; outcomes carried forward; carers' diaries no longer structurally necessary.

Detailed reference

ITEM	SUMMARY
Governing diagnosis	Operational memory is not reliably present at the point of care; the GP consultation collapses four system functions into one person under clinical consequence.
Hidden burden	Patients carry history; carers carry continuity; GPs reconstruct; receptionists route informally. These are not support roles — they are the missing system.
Cost of continuation	Consequence environment: £2.8bn NHS-wide clinical negligence payments (2023/24); 36 coroners' warnings on NHS information sharing (2024); GP exit pressure; diagnostic delay. (Sources: Diagnostic Artifact.)
Event vs state	The system is optimised for events (appointments, referrals, discharges). The patient's condition is continuous and exists between them. The system does not reliably remember between events unless a person carries the memory.
What must become true	Continuity named as a requirement; operational record separate from audit; usable history before the decision; outcomes and handovers carried forward; continuity that holds when the patient cannot carry it.

ITEM	SUMMARY
Corrected primitive	Operational continuity must be present at the point of clinical consequence, in patient-held form. Clinical authority remains with the clinician.
Minimum components	Continuity record, symptom event layer, consultation packet (under 60 seconds), outcome append, emergency access — with carer access and non-digital fallback throughout.
Move order	Truth kernel → symptom event layer → consultation packet → outcome append → emergency access → carer access → NHS recognition (Move 7 last, not first).
What must stop	Patient as default memory carrier; engagement or adoption as the success measure; routing presented as diagnosis; NHS integration as a prerequisite; audit features without retrieval gain; app treated as the official record.
Proof of success	Against baseline: GP reads the packet in under 60 seconds; patients no longer repeat their history; continuity holds when the patient cannot speak; carers' diaries no longer structurally necessary.
Recapture risk	Institutional custody transfer, audit conversion, engagement-metric optimisation, or coercive access reinstalls the failure. Patient custody and 60-second retrieval must be preserved.

CLOSING

CLOSING STATEMENT

> ***The diagnosis is not that NHS primary care lacks effort, records or digital programmes. The diagnosis is that operational memory is not reliably present where clinical decisions are made. The redesign is not another record system. It is a function reallocation: the patient holds custody, the system carries memory and preparation, and the clinician carries authority. For evidence, use the Diagnostic Artifact. For implementation, use the Full Redesign Artifact. For shared understanding, use this document.***

This Translation Artifact is derived from the Diagnostic and Full Redesign Artifacts. It introduces no new claims. For complete evidence, dependency ordering and implementation specification, refer to those documents.

Prepared by Jamie Forrester, Independent Systems Architect, Edinburgh, UK. Produced independently — not commissioned by the NHS, NHS England, DHSC or any associated body. Contact: **hello@jamieforrester.com** · jamieforrester.com